



Clinical Practice Guideline

Falls Management

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

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Introduction

This guideline outlines the procedures and responsibilities of individuals and the organisation in relation to falls and fall injury prevention, and ensures the delivery of a uniform, integrated, multidisciplinary approach with management that is consistent with best practice.

This guideline establishes the minimum standards for, falls risk assessment and subsequent management of patients at Sir Charles Gairdner Osborne Park Health Care Group (SCGOPHCG) and is aligned with the Australian Commission on Safety and Quality in Health Care Standards.

SCGOPHCG operates in accordance with the NMHS Falls Risk Management Policy.

All staff are accountable for falls management and prevention.

Definitions

Caregiver	A caregiver in this guideline is someone that provides care to a person without payment ⁵ .
Controlled/assisted fall	When a staff member attempts to minimise the impact of the fall by easing the patient's descent to the floor or by breaking the patient's fall, these events may result in an injury to the patient and or staff member. Despite this intervention, controlled falls are to be managed like any other fall.
DRSABCDE	Clinical assessment process acronym D anger, R esponse, S end for help, A irway, B reathing, C irculation, D isability, E xposure.
Fall	An event which results in a person coming to rest inadvertently on the ground or floor or other lower level^{5,6}. This definition includes: • All syncopal events • Controlled falls (the patient has not been expected to fall and required assistance to ensure their safety to a lower level. • Rolls out of bed (equipment and bed safety required a review. Patients should not be rolling out of a bed to a lower-level multiple times). Patient is suspected of having put themselves on the floor. (Injuries can still occur and need to be monitored for).
Falls Risk Identifier	 Symbol to be used when identifying a patient who is clinically judged as being at very high risk of falls.
Injurious Fall	Results in a physical injury, including head injury with or without loss of consciousness, fracture or dislocation, musculoskeletal pain, bruising, laceration, sprains, cuts, abrasions, or reduction in physical function for at least three days ^{5,6,7}
Intervention	This is a strategy identified to be implemented to mitigate a recognised risk factor.
iSoBAR	Clinical handover acronym I dentify, S ituation, O bservation, B ackground, A ssessment, and R ecommendation/ R eadback
Multifactorial Risk Assessment (MFRA)	A comprehensive process that integrates screening and assessment to identify the patients' individual risks of falling and harm from falling and the strategies required to reduce those risks ⁵ .
Multidomain Interventions	A combination of interventions tailored to the individual ⁵

1. Multidisciplinary Multifactorial Falls Risk Assessment

All patients in hospital are at high risk of falling⁵ and harm from falling. Identifying the patient's individual falls risk factors and implementing the necessary interventions are essential steps in reducing the likelihood of falls and associated injuries.

All patients are required to have a Multifactorial Falls Risk Assessment (MFRA) completed and regularly reviewed. All healthcare professionals should conduct a Multifactorial Falls Risk Assessment (MFRA) according to their specific discipline when clinically reviewing the patient. The risk factors are below. Appendix 1 indicates how the MFRA should look within the Nursing Admission and Assessment Form (NAAF). Nursing staff will also conduct an MFRA on the following occasions:

- On admission (within 4 hours)
- On transfer (to hospital or another ward) (within 4 hours)
- Post inpatient fall (within 4 hours)
- On medical condition change (as soon as possible)
- When a new NAAF Care Plan is commenced

The minimum multifactorial falls risk factors to be assessed include:

- | | |
|---|---|
| • Previous falls in the last 12 months | • Mobility impairment |
| • Previous syncope or cardiac event in last 12 months | • Vision Impairment |
| • Cognitive Impairment | • History and treatment of osteoporosis |
| • Urinary/faecal urgency/retention | • Medications that increase risk of falls |
| • Functional ability | • Previous COVID infection/exposure to prolonged period of non-activity |

Area Specific Information

The following areas complete the MFRA on the NAAF MR619.

In-patient (or multi day) wards, procedural, short stay, and outpatient areas (Short Stay Unit, Day Procedure Unit, Day of Procedure Admission, Radiology, Gastroenterology, Physiotherapy and Occupational Therapy).

[Physiotherapy](#) and [Occupational Therapy Inpatient Guidelines](#) include the addition of a stratification assessment with the MFRA.

Nuclear Medicine: In addition to completion of the MFRA on the NAAF, refer patients that require further falls assessment and intervention to the [Falls CNC](#) or Falls Clinic (pathway = OPH, Falls), prior to attending for their treatment in the Iodine Radiotherapy Therapy Room. The patient must be reassessed on admission to the ward using the NAAF .

Dialysis:

All patients must have a Falls Risk Assessment completed using the dialysis specific Falls Risk Assessment & Management Plan:

- On admission
- Three monthly
- Change in patient condition
- Reported fall at home or in the Dialysis Unit

2. Falls Risk Reduction Interventions

Interventions are multidomain and care planning is undertaken collaboratively, involving shared decision-making among the health professionals, the patient, their family and/or caregiver. This process must incorporate the individual's personal beliefs and attitudes regarding falls management⁵.

The MFRA provides risk specific interventions which can be found in [Appendix 2](#). Health professionals should use clinical judgment on identifying further multidomain risk reducing interventions that are required.

Any health professional conducting a MFRA is responsible for identifying, documenting, and implementing the interventions required to mitigate the risk of falling. For nursing, the interventions from the MFRA must be documented into the comprehensive nursing care plan.

Patients with Delirium and/or Dementia should preferably be transferred between 0700 and 1500 hours. Ward changes and room changes for this cohort should be minimised.

Prevention and reduction of deconditioning

Hospital associated deconditioning is a common problem for hospital inpatients. It leads to functional decline, loss of strength (deconditioning), increased risk of falls, complications such as pressure injuries, pneumonia, deep vein thrombosis and urinary tract infections. All have the potential to increase patients' length of stay, lead to further falls and reduce quality of life⁸. As health professionals it is important to encourage activity and exercise.

As a minimum, where clinically possible the patient should be encouraged to get dressed during the day, sit out of bed, and move as much as they are able. This includes exercises in the chair and bed as provided by the physiotherapist.

The COVID 19 pandemic has also resulted in people reducing their normal activities and increasing deconditioning. Patients are more deconditioned prior to hospitalisation than ever before. Australians are still not up to their pre pandemic levels of activity¹. The patient's pandemic experiences can increase risk of falls and harm from falls, Osteoporosis, orthostatic intolerance, and accelerated sarcopenia. WA Health released an Impact Statement on COVID on the risk of falling²⁰, which provides information and advice for health professionals to include in practice. It is vital to include in the falls risk assessment and intervention.

End PJ Paralysis as an intervention/reducing deconditioning.

 The End PJ Paralysis initiative seeks to encourage patients to get dressed and moving, promoting early functional independence, preserving dignity, and enhancing physical and psychological well-being. Additionally, it can lead to a reduction in hospital stays and the occurrence of adverse events such as falls. All ward areas are recommended to implement this programme as able.

Harm from falling is a possibility, harm from deconditioning is a certainty.

For more information visit the [End PJ Paralysis](#) page on the SCGOPHCG Falls Website.

Medications:

- Medication reconciliation by Pharmacist and Medical Officers, to include assessment and rationalisation of medicines potentially contributing to increased

falls risk, and assessment of appropriateness of medication management for complications associated with the fall.

- Suggested tools for this are [Falls Risk Increasing Drugs](#) list¹⁷ and for deprescribing – consider use of [STOPPFall](#) (Screening Tool of Older Persons Prescriptions in older adults with high fall risk)^{5,18}.

Minimum standard interventions should always be in place as appropriate for all patients.

These are listed in [Appendix 3](#).

The following are also advised for all patients

- The multidisciplinary team will facilitate referral to appropriate team members e.g., Falls CNC, (scgophcgfallscnc@health.wa.gov.au) , Physiotherapist, Occupational Therapist, Dietitian, Podiatrist, Pharmacist, to complete discipline specific MFRAs or specialty clinics and services e.g., Falls Clinic, Palliative Care, community services and those listed in the Log of Discharge Services for Falls Management at SCGOPHCG [Falls Discharge Log](#).
- Falls risk information to be included in verbal and written clinical handover, (using ISOBAR) and during attendance/transfer/discharge to other departments, facilities, or community clinics. Falls information is to be handed over to the H S A / Orderly prior to patient transfer.
- Utilise [specialised equipment](#) e.g., concave mattress, sensors, ultra-low beds, footwear, or mobility aids.
- Patients considered complex or at very high risk of falls, injury, or having two or more hospital falls should be referred to the [Falls Inpatient Service](#) (FIS) at scgophcg.fallsinpatientservice@health.wa.gov.au [formally Frequent Faller Programme] or via E-referral (Inpatient Falls). Reviews will be carried out by the Falls Clinical Nurse Consultant across SCGH/OPH or Senior Physiotherapist for the Falls Programme at OPH.
- Patients identified with cognitive impairment within the MFRA should be managed according to [SCGOPHCG Cognitive Impairment Management # CPG003](#).
- All patients being transferred to other areas must have falls risk documented in the CARPS system by the logger / requestor and the information provided to the transferring HSA/Orderly to assist in patient safety during transport.

Falls Risk Identifier

All hospital patients are considered at risk of falling; however, the Falls Risk Identifier should be utilised for those at higher risk:

- Patients who have been clinically assessed by a health professional as requiring visual reference to falls risk and/or
 - very high risk of falls
 - 1 or more inpatient falls
 - complex falls risks
 - risk of injury from a fall
- The identifier should be visible; can be put above the bed, outside of the door, (patient permission should be sought) and on the journey board.
- To reduce risk on transfer or attending imaging/investigations out of the ward/department. A laminated version should be attached to the front to the patients notes as a visual cue for staff both transporting the patient and those in the area the patient is going to. This will promote awareness and increased patient safety.
- Patient consent (where able) or communication with caregiver should be gained before displaying the sign.



C16 & C14 Falls Risk Call Bell Identifier

Every inpatient is deemed at risk of falling. It is not appropriate to have the Falls Risk call bell system set for every patient on the ward and so a criterion below has been developed to highlight those that are at very high risk.

- Previous inpatient fall
- Previous significant injury from a fall (community or hospital)
- Clinical judgement - document reasoning in health care record. Examples may be behavioural, impulsive, and/or a cognitive impairment

The yellow and black Falls Risk Identifier can be used for other patients.

Injury Risk Reduction

The very best method of preventing injury is preventing the fall. Other methods can be considered to reduce the risk of an injury.

Patients at very high risk of injury may require further interventions. Criteria for this includes:

- Has osteoporosis
- Previous fracture following a fall
- Previous Traumatic Brain Injury from a fall and on anticoagulants/antiplatelets
- Clinical judgement following health professional assessment.

Hip and Head protectors are available from some ward CNSs or from the Falls CNC

Hip Protectors

Evidence of the efficacy of hip protectors is mixed and may only have a small effect in reducing the likelihood of a fracture⁹. But they should be offered to those patients who fall frequently, have osteoporosis and/or a low body mass index. Compliance is low, mainly due to the discomfort in wearing them. However, there are patients that wear them, and it may reduce the risk of a fracture and should be considered as an available option.

Head Protectors

There are many different types but in falls management SCGOPHCG use HeadSavers®. These have had some validation testing and can reduce the risk of a head injury from a fall. Discussion and education with the patient are essential to increase comfort and compliance in wearing them.

Patient, and staff information, a care plan, and how to fit and use both products is available on the [SCGOPHCG Falls Website](#).

Isolation/Single rooms

Single rooms can increase a patient's risk of falling^{10,11,12}. Multiple studies indicate that while single rooms increase patient dignity, privacy, sleep and reduce hospital infections there is also a significant increase in falls. They also can promote loneliness and deconditioning which also increase the risk of falls. Also, whether in isolation or not the patient is out of sight.

- Consider using the [resources on the Single/Isolation Room Webpage](#)
- Frequent visual rounding

3. Patient Education

Many adults have low levels of knowledge about the causes of falls and their own risks of falling⁵. It is important to provide patients and caregivers with both verbal and written guidance on preventing falls during their hospital stay, tailored to their risks, health literacy, values and needs. This empowers older adults to assist in reducing their risks while in

hospital⁵. In cases where patients are unable to comprehend this information due to conditions such as severe cognitive impairment, it should be conveyed to a family member or caregiver on their behalf.

Patients/family/caregiver should be provided with the following minimum education:

- Orientation to surroundings and toilets
- Use of the call bell
- Why they may be at risk of falling
- That a combination of strategies is being implemented to reduce their risk
- Why supervision with ADLs maybe necessary
- How to move safely, reduce deconditioning, use aids
- How they can assist in keeping themselves safe while in hospital.

Multimodal information is available. For inpatients please print out and use the [Staying Safe During and After Hospital](#) brochure available on SCGOPHCG intranet and on the [SCGOPHCG Falls Public Website](#) and direct to watch the Free View TV patient safety video.

Other information is available on the [SCGOPHCG Falls Website](#) includes:

• Sensors: for patients	• Footwear while in hospital
• Orthostatic Hypotension	• Health Advice Following a Fall
• Amputee & Falls Risk	• Health Advice Following a Falls RAC
• Fear of Falling	

An **Inpatient Falls Education Service** is available for one-to-one education on falls risks at both sites. Referrals to [Falls Inpatient Service](#) (FIS).

Criteria for referral is listed below:

Inclusion criteria	Exclusion criteria
Falls history prior to or during admission	Cognition impaired (e.g., advanced dementia, delirium) and unable to retain education or engage appropriately during session
Willing and able to engage in falls education	Any other reason that would prevent patient from engaging meaningfully in education (e.g., behavioural issues, language barrier etc.)
	Patient is on contact/isolation precautions

Volunteer Falls Education Programme

This is run jointly by SCGOPH Falls CNC & OPH Physiotherapy for Falls Programme as part of the Inpatient Falls Education Service.

[Appendix 7](#) provides the guidelines that the volunteers operate under for this programme.

4. Post Fall Management¹⁴

Any patient admitted with a fall or experiences a fall during their admission will have their immediate and ongoing care managed by a multidisciplinary team in line with the Multidisciplinary 48 Hour Post Fall Process outlined below.

All inpatient falls must be reported in the Clinical Incident Management System (CIMS), in line with the [NMHS Clinical Incident Management Policy](#). Treating consultants are to be invited to provide third party comment for all SAC 1 and SAC 2 falls.

Further resources and information can be found on the [SCGOPHCG Post Fall page](#)

NURSING/MULTIDISCIPLINARY 48 HOUR POST FALL PATHWAY

Stop and Consider: Patients on anticoagulant, antiplatelet therapy and/or patients with a known coagulopathy (e.g., alcohol-dependent persons) are at an increased risk of intracranial, intrathoracic and/or intra-abdominal haemorrhage

IMMEDIATE POST FALL PROCEDURE

- Complete **DRSABCDE** actions
- Provide reassurance and comfort to patient and call for assistance
- Note time and date of fall
- Patient not to be moved if any physical injuries identified (unless airway is compromised) and fast track Medical Officer (MO) review within 30 minutes
- Activate Medical Emergency Team if the patient meets criteria
- Immobilise cervical spine if the patient is unconscious or reports head or neck pain
- The patient's movement to be guided by clinical assessment and the [NMHS Risk Management of Hazardous Manual Tasks Policy](#). At SCGH if there is any suspicion of spinal injury, the Jordan Frame must be utilised to move the patient. At OPH follow [Suspected Spinal Injury – Investigation and Management](#)
- Minimum investigations include blood glucose level, ECG, cognitive impairment screening using the Abbreviated Mental Test (AMT4), Rapid Clinical Test for Delirium (4AT) or Confusion Assessment Method (CAM)
- Identify immediate pre-fall symptoms e.g., dizzy, feeling unsteady, etc. and consider other investigations as indicated by the pre-fall symptoms
- Notify MO of patient fall & request review (if no apparent injury this can occur within 4 hours)
- Notify Ward/Area/Facility/ Senior Registered Nurse (SRN)/After Hours Clinical Nurse Specialist//.
- Depending on type of fall, commence observations and care delivery as follows:

WITNESSED FALL (DID NOT HIT HEAD)

- Commence neurological and baseline physical observations as per Medical/SRN's clinical judgment and document rationale

PATIENT ON ANTICOAGULANTS / ANTIPLATELETS**AND/OR****WITNESSED FALL (HIT HEAD) or UNWITNESSED FALL**

- Neurological observations:
 - Half-hourly for a minimum of 2 hours until GCS of 15 or patient considered back to their normal level of cognition achieved.
 - Continue if GCS remains < 15 or patient not considered at a normal level of cognition. Report to MO and continue as per instructions.
- If the patient has GCS of 15 or patient considered back to their normal level of cognition, then continue observations:
 - Hourly for 4 hours.
 - Two-hourly 4 hours.
 - Four-hourly for 40 hours (to make a total of 48 hours from the time of fall).
 - At 48 hours if clinically assessed as stable, and no clinical deterioration, document in the health care record and return to observations pre-fall.

PATIENT ON PALLIATIVE PATHWAY

- End-of-life patients who are no longer having observations undertaken should only resume observations after a fall if advised and documented by a consultant or senior registrar in the patient's medical record.

GOALS OF CARE CHECK

- Review patients' goals of care to identify post fall care. The patient's treating Consultant or General Practitioner must guide this.

RECOMMENDED ACTIONS WITHIN 4 HOURS:

- Medical review of patient has occurred and documented on the Post Fall Proforma for Medical Assessment MR611.
- Identify the contributing factors to the fall and implement interventions to reduce the risk of another fall/injury.

RECOMMENDED ACTIONS WITHIN 6 HOURS:

- Notify Next of Kin (NOK)/caregiver and document commencement of open disclosure processes (sticker M603) in the medical record.
- Monitor for physical, behavioural, cognitive, and clinical deterioration. Report to MO if this occurs.
- MO to have reviewed any urgent results of tests.
- Refer/notify Occupational Therapist, Physiotherapist and Pharmacist of the fall.
- Arrange referral to other health professionals as per clinical assessment.
- Ensure contributing factors to the fall have been identified and interventions implemented.
- Reassess Falls Risks, review current falls prevention interventions and implement additional interventions as required.
- Document the fall in the medical record and use the inpatient post fall sticker M499
- Complete official reporting via DATIX.
- For an injurious fall that may be considered a SAC 1 injury – notify SRN
- Record date of the patient fall on the ward journey board

RECOMMENDED ACTIONS WITHIN 24 HOURS:

- Notify Medical Officer immediately if any changes in observations including visual changes, speech/language disturbance or focal motor/sensory changes.
- Review of results of bloods, imaging, microbiology, and observations has occurred and been actioned.
- Review fall, identify and implement interventions required for the patient's falls management care.
- Patient and family/caregiver to receive information and education. Ongoing falls management care developed in partnership with patient and family/caregiver.
- The multidisciplinary team members have collaboratively discussed the fall and identified any further risks and interventions required and these have been implemented.
- Consider a structured multidisciplinary Post Fall Safety Discussion

RECOMMENDED ACTIONS WITHIN 48 HOURS:

- At 48 hours post fall, review observations and if no clinical deterioration, return to appropriate observations.
- Completion of all actions within the guidelines.
- Conduct a comprehensive review of patient care.
- Document in the inpatient record and communicate any outstanding actions and date/time completion required by multidisciplinary team members.
- Consider a structured multidisciplinary Post Fall Safety Discussion within 48 hours of the fall.

ALLIED HEALTH ASSESSMENT: OCCUPATIONAL THERAPY, PHYSIOTHERAPY, PHARMACY

- Complete assessments as per specific discipline guidelines within 2 working days of the fall

Profession Specific Guidelines for Post Falls Assessment¹⁴

- [Medical Guidelines for Post Fall Assessment \(Appendix 8\)](#)
 - Medical staff are encouraged to complete the paper version of the SCGOPHCG Medical Post Fall Proforma, which will be scanned into the DMR. If the paper version is not used, please ensure the endorsed [DMR template](#) instead
- [Physiotherapy Guidelines for Post Fall Assessment](#)
- [Occupational Therapy Guidelines for Post Fall Assessment](#)
- [Pharmacist Guidelines for Post Fall Assessment](#)

Nursing Post Fall Checklist

- The Nursing Post Fall Checklist is to be commenced immediately following an inpatient fall. (It does not currently need to be commenced for outpatient falls). It should be systematically completed to ensure all aspects of post-fall care and communication are addressed.
- Once initiated, the checklist should be placed in the bedside notes, preferably at the front of the chart, to facilitate visibility and communication across all disciplines involved in the patient's care.

- The checklist is to remain active for 48 hours post-fall, at this time the patient is to be assessed for suitability to return to pre fall observations. If nursing staff are unable to complete this assessment due to ongoing clinical concerns, escalate for medical review and a management plan.
- After this period, it may be filed in the patient's medical record.
- If the patient experiences a subsequent fall within the 48-hour period, the original checklist should be discontinued, and a new checklist commenced. In such cases, staff must ensure that the worst-case fall scenario pathway is followed and documented appropriately

Dialysis Patient Falls

The Post Fall Pathway for patients receiving dialysis requires further actions. Please refer to [Appendix 9](#).

Patients who fall in the community in the care of SCGOPHCG staff must follow the Multidisciplinary Post Falls Flowchart for Home Visiting Health Professionals – see [Appendix 4](#)

Outpatient Falls

- Follow immediate post fall procedure.
- If injured the patient must be seen in SCGH Emergency Department (ED) or if at OPH transferred to SCGH Emergency Department
- Or clinical treatment as directed by the attending medical officer in the department
- If discharged provide the leaflet' [Health Information after a Fall'\(available on intranet\) which has information for the patient about the symptoms of clinical deterioration and how to obtain assistance if concerned](#)
- Consider referral to Falls Clinic if over 65 years.
- Refer patient to [Falls CNC](#) for wellbeing check within two days of the fall.
- Report incident on Datix CIMS. Recommend follow-up review with General Practitioner

Visitor Falls

- Follow immediate post fall procedure
- If severely injured the visitor must be seen in SCGH Emergency Department (ED) or if at OPH transferred to SCGH Emergency Department
- **OPH:** For the fallen visitor call for assistance from the 311 page
- **SCGH:** If the fallen visitor is outside of the [Medical Emergency Response](#) perimeter at SCGH and is suspected of injury, an ambulance should be called for assistance.
- Refer patient to [Falls CNC](#) for wellbeing check within two days of the fall.
- Report incident on an Occupational Health & Safety Hazard Form. Recommend follow-up review with General Practitioner
- Provide the leaflet' [Health Information after a Fall'](#) (available on intranet) which has information for the visitor about the symptoms of clinical deterioration and how to obtain assistance if concerned

5. Discharge Planning

Effective communication and coordination between healthcare professionals, patients, their families, and caregivers is essential to ensure the safe discharge of patients vulnerable to falls.

The following elements should be addressed:

- Falls Management referrals to community or residential care line services should be made prior to discharge. Common referral pathways and processes are detailed in the [SCGOPHCG Falls Management Discharge Log](#).
- Education: Patients and their families or caregivers should be provided with education and training on falls prevention strategies, including information on home safety, proper use of assistive devices, and strategies for staying active and maintaining strength and balance⁵.
- Medication: Assessment and rationalisation of medicines potentially contributing to increased falls risk, and assessment of appropriateness of medication management for complications associated with the fall. Any changes (including dose adjustment, cessation, or addition of medicines) are communicated to the patient and their families or caregivers, alongside education on managing adverse effects that can contribute to falls risk.
- Home Safety: Assessment of the home environment via verbal liaison with the patient and/or carer by an Occupational Therapist to mitigate falls risks. Provide education and recommend interventions to address potential hazards. Consider including a comprehensive in-home safety assessment for high-risk patients to identify and address potential hazards in their home environment.
- Balance/Mobility problems: patients should be referred to a post hospital falls prevention exercise program.
- Discharge Summary: Patients at risk of falling or who have had an inpatient fall must have these documented in their discharge summary. Post fall incident outcomes and recommendations for follow-up are also to be documented in the discharge summary.

Patients transferring to another facility or community service must have a clinical handover from all appropriate health professionals that includes:

- The risks of falling and advised interventions
- Mobility & cognition status
- Any inpatient falls (dates) and injuries
- Confirmation that education has been provided to the patient and designated caregiver.

Patients discharged home (from either inpatient, day patient or ED) within 48 hours of a fall should receive the '[Health Information after a Fall](#)' brochure (which details information for the person/caregivers about signs of clinical deterioration and how to obtain assistance if concerned).

Patients discharged to a Residential Care Facility within 48 hours of a fall (both as an inpatient, day patient and from ED) should receive the [Health Information After a Fall for Residential Care](#) brochure which details information to the caregivers on post fall care and symptoms of clinical deterioration.

SCGOPHCG Falls Clinic

A multidisciplinary outpatient clinic that collaboratively assesses, identifies, minimises, and addresses intrinsic and extrinsic factors contributing to a patient's risk of falling. The aim is to minimise the risk of falls and associated harm by working in partnership with the patient and carer to deliver comprehensive information and management strategies, including aspects like osteoporosis and personalised exercise plans.

The clinic manages and reviews its assessments and interventions according to the SCGOPHCG Falls Guidelines CPG and on community best practice evidence which is not included in the inpatient guideline.

Patients can be referred by a health professional while an inpatient, or by the General Practitioner.

6. Health professional knowledge of falls, their roles and education resources

All staff working with patients must receive education in falls management relevant to their role⁵.

Knowledge should include as a minimum, risk factors for falling, common interventions, and post fall care. Information and education packages can be found on the [SCGOPHCG Falls Management Intranet Website](#).

Specific requirements:

All staff:

- On induction and orientation to work area
- Refresher updates every two years.

Nursing:

- Falls Self-Directed Learning Package with three months of commencing a new area/ward/hospital
- Manual handling education

Medical Staff:

- Information can be found on the [Medical Education Unit SharePoint Site](#)

Allied Health: For information

- Contact Head of Department
- [Role of Allied Health in Falls Management](#)
- [SCGOPHCG Falls Management Intranet Website; Allied health](#)
- [Stay On Your Feet® Falls Prevention Program WA \(injurymatters.org.au\)](#)
- **Physiotherapy and Occupational Therapy:**
 - New staff orientation/induction - falls management education.
 - Annual mandatory manual handling training.
 - Annual department professional development - falls audit report and education.

Compliance, monitoring and evaluation

The SCGOPHCG Falls Management Working Group (FMWG) will monitor compliance with this policy via:

- Ward auditing results (CENSAS)
- SAC 1 and SAC 2 investigations
- Post Fall Auditing undertaken by the Falls Clinical Lead and FMWG

Collated data will be presented six monthly to the FMWG for review and action.

Results will also be reported up to the Comprehensive Care Standard 5 Committee and included in annual reports.

Related Documents

- [NMHS Falls Risk Management Policy](#)
- [NMHS Clinical Incident Management Policy and Procedure](#)

- [NMHS Risk Management of Hazardous Manual Tasks](#)
- [SCGOPHCG Communicating for Safety Policy](#)
- [SCGOPHCG Sensor Mat Use Multidisciplinary Guidelines](#)
- [SCGH Footwear For Inpatient Safety](#)
- [SCGOPHCG Cognitive Impairment Policy](#)
- [SCGOPHCG Interpreter Services Policy](#)
- [WA DOH Post Fall Multidisciplinary Management Guidelines 2023](#)

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Appendix 1: Integrated Multifactorial Falls Risk Assessment (MFRA)⁵

Yes/No						
Mobility & Functional Risks						
Medications/Medical Condition Risks						
Cognitive Risks						
Continence/Elimination						

Appendix 2: Basic Falls Interventions Following MFRA⁵

Mobility & Function Interventions	
- Had a fall in the last 12 months - Requires supervision/assistance with mobility/transfers/or uses a gait aid - Requires supervision/assistance with ADLs - Has poor balance/coordination/abnormal posture/movement disorder - Exposure to long periods of inactivity	Conduct a simple mobility assessment if no physiotherapist available
	Refer to physiotherapist for a comprehensive mobility assessment
	Confirm with the patient and document level of assistance/gait aids required
	Always ensure gait aids within reach of the patient
	Document a mobility goal for the patient if appropriate
	Complete Lying and Standing BPs and implement care as per SCGOPHCG Orthostatic Hypotension Guideline
	Referral to Occupational Therapist for functional assessment
	Has safe footwear in line with SCGOPHCG Footwear for Safety Policy
Medications/Medical Condition Interventions	
- Has a medical history of osteoporosis - Has had recent history of syncope or cardiac event - Complains of dizziness on standing/known or at risk of postural hypotension - Takes >8 medications of any sort, and/or medications that include antidepressants, antipsychotics, antipsychotic, benzodiazepines, anticholinergic medication, opioids. - COVID 19 infection, Long Covid, physical inactivity - Has a vision impairment	Complete Lying and Standing BPs and implement care as per SCGOPHCG Orthostatic Hypotension Guideline
	Educate patient to stand up slowly and wait until dizziness resolves before mobilising
	Consider Injury Reduction eg Hip/Head protectors
	Liaise with Medical Officer (MO) or Pharmacist for review/deprescribing/rationalisation of medicines potentially contributing to falls risk
	Liaise with Medical Officer. Complete and review by Medical Officer review of ECGs, CTs, EEG, Holter monitors
	Document type of visual impairment and extent of visual field
	Ensure adequate lighting, minimal shadows in the room when mobilising
	Ensure daily objects are within visual field of the patient
	Ensure glasses are clean and within reach
	Use vision impaired sign if patient consents
Cognitive Interventions	
Has a cognitive impairment e.g., delirium, dementia	Patient to always be supervised with mobility and ADLs
	Complete 4AT
	Appropriate interventions from the SCGOPHCG Cognitive Impairment Management CPG
	Complete Lying and Standing BPs and implement care as per SCGOPHCG Orthostatic Hypotension Guideline
	Use of Sensor mats , ultra-low beds, versacare beds if patient impulsive and declines or unable to use a call bell
Concern about falling	Refer to Occupational Therapist
Continence/Elimination Interventions	
Has urinary/faecal incontinence/urgency	Complete a continence assessment & optimise management. Consider Medical referral if required.
	Document access and equipment required to toilet
	Obtain equipment ready for patient to use
	Document if supervision required
	Document any cultural requirements
Environmental Interventions	
Environmental hazards	Remove or modify all hazards in the environment that may increase risk of a fall.

Appendix 3: Minimum Falls Interventions:

Minimum interventions for all patients to reduce risk of falls

- Provide ongoing orientation for patient to bed area, toilet facilities, ward, and ward routine
- Demonstrate the use of call bell, ensure it is in reach and that they can use it effectively
- Instruct patients to call for assistance when getting out of bed/mobilising
- In partnership with the patient and informal caregiver provide falls prevention information, tailored to their risks, health literacy, values and needs
- Ensure frequently used items including mobility aids are within easy reach of patient
- Encourage patient to use their aids such as glasses or hearing aids
- Ensure the chair and bed height / position are suitable for the patient's needs (i.e., the patient's feet need to be on the ground with the knees slightly below the hip)
- Minimise prolonged bed rest as it contributes to negative cardiovascular and muscle effects that may lead to falls
- Place IV pole and all other devices/attachments on exit side of bed
- Remove clutter and obstacles from room
- Footwear in line with the [SCGOPHCG Footwear for Safety](#) Policy
- Ensure brakes are correctly applied to all bed, wheelchairs, commodes when stationary
- Communicate patient at risk of falls at handover

Appendix 4: Home Visiting Post Falls Flowchart¹⁴

Patient Falls at Home

(includes witnessed or unwitnessed falls at home or in community in last 72 hours)

Assess

- Reassure patient
- Perform D.R.S.A.B.C.D
- Administer First Aid within skill limit
- Check for injury(*)
- Move patient if required for safety(*)
- Make comfortable until help arrives

Escalate Care?

- Loss of consciousness/ Evidence of seizure
- Noted change in cognition/behaviour(*)
- Hit head or visible evidence Head Injury.
- Signs of Injury
- Pain or symptoms of clinical concern
- Use of Anticoagulation medication(*)
- Unable to move from floor safely or without assistance(*)

Seek Assistance

- General Practitioner / Locum service
- Paramedic/ Ambulance(*) CALL 000
- Emergency Department
- Hospital/ ED Avoidance Programs
- Local Health Service Medical Officer
- Anticoagulation Nurse Practitioner(*)

*See over for additional information



Government of **Western Australia**
Department of Health

Multidisciplinary Post Fall Flowchart for Home Visiting Health Professionals

Good Practice Points

Remember

- Provide patient/ carer "**Health advice following a Fall-Community**" brochure. Review information as appropriate.
Visit: https://www2.health.wa.gov.au/Articles/F_f/Falls-prevention-and-management-in-WA
- Document fall in patients medical file(PTO for documentation prompts and criteria for completion of "CIMS form")*
- Consider referral for management of their ongoing falls risk.
Visit: <https://www.stayonyourfeet.com.au/health-professionals/referrals-to-health-professionals/> or access it directly here **SOYFWA**

Communication

- Advise your immediate supervisor/ manager of incident
- Obtain patient consent as able prior to initiating care
- Contact Next of Kin, Carer/ Care provider
Alert them to fall. Request visit I review if possible
- Contact patients GP. *Alert them to fall*
- Liaise with Multidisciplinary team
- Contact patient later in the shift or next day to check well-being.

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Government of Western Australia
Department of Health

Multidisciplinary Post Falls Flowchart for Home Visiting Health Professionals: Accompanying Information

This flowchart should be used by home visiting health professionals (HP) to guide their assessment and management of patients who have fallen at home within the preceding 72 hours *and not received medical assessment*. It is not intended to take away from clinical judgment. HP should work within their Scope of Practice when managing the patient who has fallen. If there are any concerns with acute management of a fall or ongoing management of their falls risk, they should be referred to an appropriate health service as outlined in the flow chart. For long term management of falls risk refer to the [Stay on Your Feet Health Professional](#) portal for a list of services available in WA. Refer to [WA Post Falls Guidelines 2023](#) for further information.

DATIX CIMS notification. A CIMS form should be completed when a fall occurs whilst the HP is present *or* the patient is on floor when the HP arrives at the patient's house. Notification of a clinical incident is made online using the Datix CIMS application or if electronic access is not available via a hard copy clinical incident form (contact your line manager). Further details re CIMS reporting can be found in the [WA DOH CIMS Policy Pack \(CIMS Policy 2019; CIMS Guidelines 2019; CIMS Toolkit 2019\)](#).

Documentation Prompts. Circumstances of fall (what, how, when), associated symptoms occurring at time of fall (e.g. dizziness, light-headedness, syncope, vertigo, weakness, incontinence, confusion / disorientation), associated falls related injuries, was the fall witnessed, environmental hazards, medical / nursing / allied health / First Aid treatment required and delivered. Include any communication performed to report the fall.

Checking for Injury. Encourage the client to stay calm and avoid moving or trying to stand whilst you undertake your assessment. Is the patient complaining of pain in the neck or head region? Can the patient move their upper and lower limbs without pain? Is there any new bruising, swelling or deformity? Has the patient's mobility changed as compared prior to the fall? Has the patient's conscious state (e.g. alertness, arousal, appropriately following commands) or cognition (e.g. memory, reasoning and orientation) changed? *Note:* Any alteration in a patient's conscious state or cognition may impact on their capacity to make decisions regarding their ongoing care including the need for further medical intervention after a fall. HP should consider further assessment if there are concerns about an acute change in the patients cognition (e.g. Delirium) using a recognised tool such as the [4AT](#) (inclusive of AMT4) or communicate with the clients GP or primary carer for further clarification regarding the patient's baseline cognitive level if there is any uncertainty.

Manual Handling - moving patient's safely. All staff are requested to follow local OSH guidelines during lifting and transfer of patient loads. Staff should consider their individual training and clinical skills prior to deciding whether to assist a patient on the floor. Staff should only consider moving a patient from the floor if:

- Risk assessment indicates that they are able to assist without injury to the patient or themselves at all times.
- Safe manual handling techniques and postures are maintained. Please refer to the local OSH Manual Handling guidelines for a description of an appropriate technique in a domiciliary setting. Staff can download the [Stay on Your Feet Up from Floor Poster \(Arms or Knees\)](#) for people able to perform the floor transfer independently, via prompting or with supervision.
- The patient needs to be removed from immediate danger.

When to call the ambulance. If a person falls at home or in the community and cannot get up independently or with minimal assistance and / or if urgent medical assistance is required an ambulance should be called (please refer to local policy and guidelines). In addition, consideration should be given to other assessment findings such as pain of clinical concern (including new neck or head pain), altered consciousness, confusion, significant injury requiring medical attention or clinical judgement suggests that there is concern over the patients wellbeing.

Communication. Staff should attempt to make contact with the patient's GP and primary carer to alert them to the fall, consequences of the falls and any interventions delivered or required. Patient consent should be obtained where possible. Staff should ensure that the patient has appropriate capacity to make an informed decision (*Refer to Checking for Injury above*) in the event that they refuse medical attention or refuse to have their GP / primary carer alerted to the fall. If the patient has capacity to make a decision, staff should abide by the patients request. However if the staff member is uncertain about the patients capacity either due to pre-existing medical conditions or as a result of the fall (e.g. hit head) then contacting the patient's GP to assist in determining capacity and the need for medical assistance may be justified if it is in the patients best interest. Staff should seek further assistance by contacting their line manager.

The fallen patient who has Coagulopathy or is on prescribed Antiplatelet and or Anticoagulant medication. Patients who are coagulopathic, on antiplatelet and or anticoagulant medications are at an increased risk of bleeding related injuries after a fall. Common medications include: Warfarin, Clexane, Rivaroxaban, Dabigatran, Apixaban, Aspirin Clopidogrel, Ticagrelor, Asasantin SR, *Dalteparin (A detailed list has been produced by QLD Health)*. If the person does not require transport to Hospital, an urgent visit to their GP is recommended due to the risk of undiagnosed internal bleeding. If patient is under the management of an Anticoagulation Service contact the service provider for further advice regarding management.

DISCLAIMER: This document has been developed with care but may not be appropriate for all clinical scenarios. Clinical assessment along with medical assistance and advice must be sought if in any doubt.

Last updated April 2023

Further information can be obtained from
Falls.ManagementWA@health.wa.gov.au

Appendix 5: Common Falls Risk Increasing Drugs (FRIDs)¹⁷

FALLS RISK INCREASING DRUGS (FRIDs)			
Psychotropic medicines			
<u>Antidepressants</u>	• Monoamine oxidase inhibitors • SSRIs • SNRIs • Other antidepressants • Tricyclic antidepressants • Comparative adverse effects	<u>Antipsychotics</u>	All Comparative adverse effects
<u>Anxiolytics/ Sedatives/ Hypnotics</u>	• Benzodiazepines • Z-drugs • Other: Suvorexant		
Cardiovascular medicines			
<u>Antiarrhythmics</u>	• Amiodarone • Digoxin • Flecainide • Sotalol	<u>Antihypertensives</u>	• ACE inhibitors • Sartans • Beta-blockers • Calcium channel blockers • Thiazide & Related Diuretics • Other antihypertensives • Clonidine • Methyldopa • Prazosin
<u>Heart failure medicines</u>	• Aldosterone antagonists • Loop diuretics • Other HF medicines • Ivabradine • Sacubitril with valsartan • Sodium-glucose co-transporter 2 inhibitors - Australian Medicines Handbook (health.wa.gov.au)	<u>Nitrates and other vasodilators</u>	• Nitrates • Pulmonary hypertension medicines • Other vasodilators
Other medicines			
<u>Anticholinergics</u>	• Numerous drugs have anticholinergic effects • Hyoscine (butylbromide & hydrobromide) • Inhaled bronchodilators	<u>Antihistamines</u>	• Sedating antihistamines • Less sedating antihistamines
<u>Parkinsonism Medicines</u>	• Dopamine agonists • Monoamine oxidase type B inhibitors • Anticholinergics • Benztropine • Trihexyphenidyl	<u>Opioids</u>	All, alone or in combination
<u>Beta-blocker eye drops</u>	• Betaxolol • Timolol	<u>Genitourinary</u>	• Selective alpha blockers • Phosphodiesterase inhibitors • Anticholinergics
<u>Hypoglycaemics</u>	• Sulfonylureas • Insulins • Sodium-glucose co-transporter 2 inhibitors - Australian Medicines Handbook (health.wa.gov.au)	<u>Other</u>	• Prochlorperazine

NB: This list is not exhaustive. Please consider consulting other medication reference text. More information can be found at <https://www.nswtag.org.au/polypharmacy-qum-indicators-and-resources/>

Appendix 6: Deprescribing guidance¹⁸

To support clinicians, the STOPPFall deprescribing tool was developed and recommended by the World Guidelines for Falls.⁵ Please refer to the tables below.

Deprescribing guidance for STOPPFall items ¹⁸			
	Fall-risk assessment: In which cases to consider withdrawal?	Is stepwise withdrawal needed?	Monitoring after deprescribing
Always	If no indication for prescribing		- Fall incidence and change in symptoms e.g., OH, blurred vision, dizziness - Organise follow-ups on individual basis
Benzodiazepines (BZD) and BZD-related drugs	- If daytime sedation, cognitive impairment, or psychomotor impairments - In case of both indications: sleep and anxiety disorder	In general, needed	- Monitor: anxiety, insomnia, agitation - Consider monitoring: delirium, seizures, confusion
Antipsychotics	- If extrapyramidal or cardiac side effects, sedation, signs of sedation, dizziness, or blurred vision - If given for BPSD or sleep disorder, possibly if given for bipolar disorder	In general, needed	- Monitor: recurrence of symptoms (psychosis, aggression, agitation, delusion, hallucination) - Consider monitoring: insomnia
Opioids	- If slow reactions, impaired balance, or sedative symptoms - If given for chronic pain, and possibly if given for acute pain	In general, needed	- Monitor: recurrence of pain - Consider monitoring: musculoskeletal symptoms, restlessness, gastrointestinal symptoms, anxiety, insomnia, diaphoresis, anger, chills
Antidepressants	- If hyponatremia, OH, dizziness, sedative symptoms, or tachycardia/arrhythmia - If given for depression but depended on symptom-free time and history of symptoms or given for sleep disorder, and possibly if given for neuropathic pain or anxiety disorder	In general, needed	- Monitor: recurrence of depression, anxiety, irritability, and insomnia - Consider monitoring: headache, malaise, gastrointestinal symptoms
Antiepileptics	- If ataxia, somnolence, impaired balance, or possibly in case of dizziness - If given for anxiety disorder or neuropathic pain	Consider	- Monitor: recurrence of seizures - Consider monitoring: anxiety, restlessness, insomnia, headache
Diuretics	- If OH, hypotension, or electrolyte disturbance and possibly if urinary incontinence - possibly if given for hypertension	Consider	Monitor: heart failure, hypertension, signs of fluid retention
Alpha-blockers (AB) used as antihypertensives	If hypotension, OH, or dizziness	Consider	- Monitor: hypertension - Consider monitoring: palpitations, headache
AB for prostate hyperplasia	If hypotension, OH, or dizziness	In general, not needed	Monitor: return of symptoms
Centrally-acting antihypertensives	If hypotension, OH, or sedative symptoms	Consider	Monitor: hypertension

Sedative antihistamines	• If confusion, drowsiness, dizziness, or blurred vision • In case of all indications: hypnotic/sedative, chronic itch, allergic symptoms	Consider	• Monitor: return of symptoms • Consider monitoring: insomnia, anxiety
Vasodilators used in cardiac diseases	• If hypotension, OH, or dizziness	Consider	• Monitor: symptoms of Angina Pectoris
Overactive bladder and incontinence medications	• If dizziness, confusion, blurred vision, drowsiness, or increased QT-interval	Consider	• Monitor: return of symptoms
Abbreviations: BPSD = Behavioural and Psychological Symptoms of Dementia; OH = Orthostatic Hypotension			

An online interactive version of the STOPPFall deprescribing tool may be available (depending on settings): <https://www.eugms.org/research-cooperation/task-finish-groups/frid-fall-risk-increasing-drugs.html>

Appendix 7: Volunteer Inpatient Falls Education Program

The volunteers for this program operate under the [NMHS Volunteer Policy](#).

The Volunteer Inpatient Falls Education Program at SCGH and OPH aims to provide one on one falls education to the patient tailored to their specific falls risks. It encourages an adult learning style, with the use of discussion and written material.

The education is guided by a booklet called “Keeping safe and preventing falls in hospital” or the “Falls prevention for amputees” leaflet.

On employment to the program the volunteer (who will already have a health professional background) receives training based on the Safe Recovery Programme.¹⁹ This includes falls risk factors, common interventions, learning styles, process, potential problems and how to troubleshoot, the booklet itself, documentation in the patient record, key people at OPH and how to deliver the education using Teachback. The volunteer will shadow the Falls CNC delivering the education, and then deliver education under supervision until deemed competent.

Patients can be identified via referral to the OPH Senior Physiotherapist in falls management or SCGOPHCG Falls CNC, attend the ward and discuss with ward staff or identify patients from the letters FE (Falls Education) on the Journey Board.

The volunteer will check with nursing staff if the patient can be seen and then gain consent of the patient for the discussion. At the end of the session the Volunteer (or the Falls CNC on their behalf) will document in the patient’s health care record that falls education has been provided and put a line through FE on the Journey Board.

The volunteer is not employed or acting as a health professional and must not provide clinical advice other than in relation to falls prevention as laid out in the booklet. The volunteer will ensure that the multidisciplinary team are made aware of any concerns or problems relating to the patient.

The volunteer will provide the URN of the patients seen to the SCGOPHCG Falls CNC for record keeping, any follow up and feedback required.

Criteria for referral

Inclusion criteria	Exclusion criteria
Falls history prior to or during admission	Cognition impaired (e.g., advanced dementia, delirium) and unable to retain education or engage appropriately during session.
Willing and able to engage in falls education	Any other reason that would prevent patient from engaging meaningfully in education (e.g., behavioural issues, language barrier etc.)
	Patient is on contact/droplet precautions

Inpatient Volunteer Falls Education Programme Summary

How to complete the education session:

- Check 3pt ID and consent. Introduce yourself including role and reason for visit. Provide estimate of time the session will take.
- Provide a copy of the relevant patient handout to guide conversation:
 - Patients (over 65) – *'Keeping Safe and Preventing Falls in Hospital'*
 - Amputee patients – *'Falls prevention information for lower limb amputees'*
- Use the following questions and information in the handouts to prompt patient:
 - Have you had many falls? Why do you think you have fallen?
 - Why do you think you are you at risk of falling?
 - How do you think you could prevent another fall?

Common reasons for falls:

- Fainting/BP drop
- Loss of balance/not using walking aid appropriately
- Hurrying/rushing e.g., to toilet
- Poor lighting e.g., overnight
- Reduced blood sugar or dehydration e.g., not eating/drinking, diabetic
- Medication and associated effects e.g., drowsiness
- Trip hazards/poor footwear

Education volunteers can provide:

- Use walking aid as directed
- Use the call bell – ask for help, wait for help
- Keep belongings in easy reach e.g., glasses, hearing aids
- Wear safe footwear / non-slip socks
- Sleep hygiene e.g., stay awake in the day, sleep at night
- Stay hydrated and eat well
- Take time with transfers/walking – plan the task, concentrate (minimise distractions)
- If alcohol history is mentioned – discuss associated falls risk
- Amputees –
 - Take care with transfers – plan task/appropriate positioning/use brakes etc.
 - Can provide education on risk of injury to residual limb with fall/delayed healing etc.

Minimum inpatient falls interventions:

1. Ongoing orientation to bed / bathroom / ward
2. Demonstrate use of call bell, ensure it is in reach and they can use it effectively
3. Ensure frequently used items including mobility aids are within easy reach of patient
4. Encourage patient to use their aids e.g., glasses, hearing aids
5. Adjust bed and chair to appropriate height for patient
6. Minimise prolonged bed rest (deconditioning increases risk of falls)
7. Place IV pole and other attachments on exit side of bed
8. Remove clutter and obstacles from room
9. Provide adequate lighting according to patient activities/needs
10. Encourage patient to consume adequate fluids and nutrition
11. Optimise footwear
12. Educate that all inpatients are at increased risk of falling due to injury/illness/medications

Appendix 8: Medical Assessment Post Fall¹⁴

Complete assessment within 30 minutes for an injurious fall within four hours for all other falls. Use the Post Fall Proforma for Medical Assessment MR611

Medical assessment best practice completed as minimum:

Protocol

- Every patient experiencing a fall in hospital requires a timely medical review (urgent if deteriorating, within 30 minutes if injured, and within 4 hours for most other falls).
- Responding to the fall incident requires the provision of immediate first aid, longer-term care, and active addressing of falls risk factors to prevent future falls.

History

- Talk to the patient about the fall and symptoms arising from the fall.
- Review medical entries in the patient's health care record and medication chart to identify factors that may put the person at risk of falling, or of having an injury from the fall.
- Establish the patient's baseline mobility and cognitive state and determine whether it has changed post fall.
- Document if the patient is taking any anticoagulant or antiplatelet medicines e.g. warfarin, enoxaparin (Clexane®), heparin, apixaban (Eliquis), rivaroxaban (Xarelto), dabigatran (Pradaxa), prasugrel (Effient) and ticagrelor (Brilinta), fondaparinux, lepirudin, bivalirudin, aspirin, clopidogrel, dipyridamole (Asasantin®) or other anticoagulant/antiplatelet medication.
- Patients with chronic liver disease or haematological disorders may also be coagulopathic.

Examination

- Examination should always take place, even if you must wake the patient.
- The examination should identify any injury sustained. When examining a patient, be aware that they may not draw attention to all their injuries (particularly if cognitively impaired).
- Do not allow the patient to be moved until head, cervical spine and hip injuries have been ruled out. Spinal precautions must be used if the patient has GCS <13 or a neck injury is suspected.
- The examination should also seek to identify the immediate underlying causes of the fall (infection, arrhythmia, stroke, hypotension, other acute illness).
- The examination should include:
 - Check pulse and blood pressure (when appropriate check postural drop).
 - Assess level of consciousness and document Glasgow Coma Scale.
 - Talk to the patient – assess for confusion (delirium or dementia). Document AMT4 (age, DOB, current year, place).
 - Examine the head, neck, spine, hips, and limbs to identify sites of tenderness/swelling/deformity (for example a shortened, externally rotated leg may indicate a hip fracture).
 - Neurological examination including speech, pupil size, eye movements, facial asymmetry, power, sensation, and plantar responses.
 - If there are no obvious features of hip fracture, ensure hip range of movement is pain free, and as soon as is practicable ensure weight bearing is also pain free.
 - Assess post-fall mobility.

Investigations

- Order relevant investigations and ensure the results are checked and documented in the patient's health care record.
- Exclude intracranial haemorrhage and fractures.

1. Is a CT head scan required?

a). If the patient has hit their head?¹⁶

This decision should be individualised and based on their risk of injury. There is no specific research determining the optimal pathway for inpatients. The National Institute for Health Care and Excellence (NICE) guidelines developed for Emergency Departments provide useful criteria for clinicians to assist decision-making.

These are reproduced below in Figures 1 and 2. Deterioration in neurological observations undertaken by nursing staff is a trigger for CT scanning.

Fig 1. When to perform a CT head scan within 1 hour¹⁶

For adults who have sustained a head injury and have any of the following risk factors, perform a CT head scan within one hour of the risk factor being identified:

- GCS less than 13 on initial assessment.
- GCS less than 15 at 2 hours after the injury on assessment.
- Suspected open or depressed skull fracture.
- Any sign of basal skull fracture (haemotympanum, 'panda eyes', cerebrospinal fluid leakage from the ear or nose, Battle's sign).
- Post-traumatic seizure.
- Focal neurological deficit.
- More than one episode of vomiting.

Fig 2. When to perform a CT head scan within 8 hours¹⁶

For adults with any of the following risk factors who have experienced some loss of consciousness or amnesia since the injury, perform a CT head scan within 8 hours of the head injury:

- Age 65 years or older.
- Any history of bleeding or clotting disorders.
- Current anticoagulation treatment.
- Dangerous mechanism of injury (a pedestrian or cyclist struck by a motor vehicle, an occupant ejected from a motor vehicle or a fall from a height of greater than one metre or five stairs).
- More than 30 minutes' retrograde amnesia of events immediately before the head injury.

b). If patient had a witnessed fall and did NOT hit their head and they do NOT need a CT head scan?

- CT Head required if signs of neurological impairment develop.

c). Unwitnessed fall with no signs of head injury?

Consider CT head scan within 8 hours if:

- Cognitively impaired.
- Neurological deterioration on nursing observations.
- On current anticoagulant treatment.

Whether a CT head scan will alter patient management and patient/caregiver preferences should be considered; for example, would the patient be considered appropriate for neurosurgical intervention? This dialogue should be documented in the patient's health care record and discussed with the treating specialist.

2. Is a CT cervical spine scan required?¹⁶

a) For patients with a head injury:

NICE guidelines¹⁶ make the following recommendations in relation to cervical CT requests:

Fig 3. Risk factors indicating CT cervical spine within 1 hour¹⁶: A cervical spine CT should be arranged within one hour for all adults who have sustained a head injury and have any of the following risk factors:

- GCS less than 13 on initial assessment.
- The patient has been intubated.
- Plain X-rays are technically inadequate (for example the desired view is unavailable).
- Plain X-rays are suspicious or abnormal.
- A definitive diagnosis of cervical spine injury is needed urgently (for example before surgery).
- The patient is having other body areas scanned for head injury or multi-region trauma.
- The patient is alert and stable, there is clinical suspicion of cervical spine injury and any of the following apply:
 - Age 65 years or older.
 - Dangerous mechanism of injury (fall from a height of greater than 1 metre or 5 stairs).
 - Focal peripheral neurological deficit.
 - Paraesthesia in the upper or lower limbs.

Fig 4. Assessing range of neck movement safely¹⁶

For adults who have sustained a head injury and have neck pain or tenderness but no indications for a CT cervical spine scan, perform three-view cervical spine X-rays within one hour if either of these risk factors are identified:

- It is not considered safe to assess the range of movement in the neck.
- Safe assessment of range of neck movement shows that the patient cannot actively rotate their neck to 45 degrees to the left and right.

Be aware that in adults who have sustained a head injury and in whom there is clinical suspicion of cervical spine injury, range of movement in the neck can be assessed safely before imaging only if there are no high-risk factors and at least one of the following low-risk features apply. The patient:

- Was involved in a simple rear-end motor vehicle collision.
- Is comfortable in a sitting position.
- Has been ambulatory at any time since injury.

Treatment

- Implement treatment as appropriate (for example resuscitation, immobilisation, pain relief).
- If patient has sustained significant injuries, inform the patient's consultant (or on call consultant after hours). If intracranial haemorrhage is confirmed, also urgently consult the neurosurgical registrar or consultant on call.
- If patient is unstable, return as often as deemed clinically appropriate to review.
- Implement appropriate actions to prevent a recurrence of a fall and communicate these to relevant staff.
- Review for antiplatelet and/or anticoagulant therapy or [Falls Risk Increasing Drugs](#)¹⁷. If clinical evidence for head injury, withhold anticoagulants until CT head scan is available.
- Inform the relevant medical team for follow-up.
- In the case of significant injuries, the doctor should inform the Next of Kin (NOK) if patient consents. Nursing staff will inform NOK about less serious falls.
- Consider referral to relevant members of the multidisciplinary team for further assessment and interventions as required

Stop and consider

- Have head, cervical spine and hip injuries been adequately ruled out? Do not allow patient to be moved until you have done so.
- Spinal precautions need to be used if the patient has GCS <13 or a neck injury is suspected.
- Has a new medical problem, for example sepsis, been adequately ruled out or treated?
- Does the patient have delirium or dementia, and is management in accordance with best practice?

Document and handover

- Documentation in the patient's health care record is vital.
- Use the Post Fall Proforma for Medical Assessment MR 611.
- Communicate with relevant staff.
- If you have any doubts about appropriate investigations and management, contact the appropriate senior medical person and after hours, the afterhours registrar or medical officer on call.

Medical treating team within 48 hours of the fall:

- Follow up any urgent investigations.
- Review medications e.g., sedatives – consider weaning off if clinically appropriate
- Collaborate with the multidisciplinary team to identify all the patient's falls risk factors.
- Formulate an individualised management plan to address these.
- Assess for delirium and manage.
- Ensure the fall is mentioned on the discharge summary.

Discharge

The patient and caregiver should be provided with written and verbal information about falls. Discuss with the patient and caregiver about the risks of clinical deterioration, further falls and ensure that they have understood.

If discharged within 48 hours of a fall a ["Health Information After a Fall"](#) leaflet and discussion should be provided to ensure that the patient and caregiver have an understanding of the signs and symptoms of any clinical deterioration and where to seek help.

***Note: If the following occur, the fall must be reported to the coroner:**

- a fall that resulted in the patient's death.
- a fall that was sustained within 48 hours prior to the patient's death.

Appendix 9: Dialysis Falls

Any patient who experiences a fall during admission in the dialysis unit will have their immediate and ongoing care managed by a multidisciplinary team in line with Nursing/Multidisciplinary 48-hour Post Fall Pathway with additions as follows:

Immediate Post Fall Procedure	All actions <u>plus</u>: Cease/do not commence dialysis anticoagulation medication (e.g., Heparin). Dialysis anticoagulation medication may be administered following MO review and confirmation of no contraindications. If injured, the patient must be seen in SCGH ED or receive clinical treatment as directed by the attending MO in the department.
Recommended Actions with 4 hours	All actions must occur <u>prior to patient discharge</u> from dialysis unit except Multidisciplinary team requirements which are recommended within 24 hours of patient fall.
Recommended Actions with 6 hours	
Recommended Actions within 24 hours	

Any patient presenting for dialysis who has experienced a fall since previous dialysis treatment will be managed as follows:

Patient presents within 24 hours post fall	
Head strike	Notify shift coordinator. Neurological observations: Nursing/Multidisciplinary 48-hour Post Fall Pathway Immediate Post Fall Procedure observations (0 hrs = time and date of fall). Requires MO review or transfer to ED prior to commencement of dialysis. MO or senior nurse review required prior to transfer to ED. Do not commence dialysis anticoagulation medication. Dialysis anticoagulation medication may be administered following MO review and confirmation of no contraindications.
No head strike	Requires MO review. If MO is not immediately available patient must have senior nurse review before commencement dialysis. Do not commence dialysis anticoagulation medication. Dialysis anticoagulation medication may be administered following MO review and confirmation of no contraindications.
Patient presents with 24 – 48 hours post fall	
Requires MO review. If MO is not immediately available patient must have senior nurse review before commencement dialysis.	

Recommendations for all patients who have experienced a fall during or between dialysis treatments include provision the education leaflet: [Health Information After a Fall](#) and referral to Falls Clinic or [Falls CNC](#).